

## STROKE UNIT DELIRIUM SCREENING FORM

Companion to Stroke Unit Delirium Screening Protocol — Use with 4AT, CAM, ICDSC, or CAM-ICU per protocol Section 5.2

|                      |                                           |                                      |
|----------------------|-------------------------------------------|--------------------------------------|
| Patient Name:        | MRN:                                      | DOB:                                 |
| Admission Date/Time: | Stroke Type (Ischemic / ICH / SAH / TIA): | Admission NIHSS:                     |
| Unit / Bed:          | Attending / Stroke Team:                  | Post-tPA/Thrombectomy? Y / N — Time: |

### 1. Reference Neurological / Cognitive Exam (Complete Once on Admission; Update if Deficits Evolve)

Per protocol Section 5.1 — this establishes the stroke-related baseline against which every subsequent delirium screen must be interpreted. A screen is only truly positive when it reflects a change superimposed on this baseline.

| Domain                                                   | Findings |
|----------------------------------------------------------|----------|
| Pre-stroke baseline mentation (patient/family report)    |          |
| Aphasia (type: expressive / receptive / global / none)   |          |
| Neglect (side, if present)                               |          |
| Hemianopia / visual field deficit                        |          |
| Dysarthria                                               |          |
| Baseline level of consciousness / RASS at rest           |          |
| Other relevant deficits                                  |          |
| Reference exam completed by (name/credential, date/time) |          |

### 2. Screening Tool Quick Reference

#### 2.1 4AT (Rapid Screen — use with caution if known aphasia/neglect; confirm positives with CAM)

| Item                                      | Scoring                                                              | Score                                                                            |
|-------------------------------------------|----------------------------------------------------------------------|----------------------------------------------------------------------------------|
| Alertness                                 | Normal = 0; Mild sleepiness >10 sec or agitation = 4                 | <input type="checkbox"/> 0 <input type="checkbox"/> 4                            |
| AMT4 (age, DOB, place, current year)      | No mistakes = 0; 1 mistake = 1; ≥2 mistakes = 2                      | <input type="checkbox"/> 0 <input type="checkbox"/> 1 <input type="checkbox"/> 2 |
| Attention (months backward from December) | ≥7 months correctly = 0; starts but <7 / refuses = 1; untestable = 2 | <input type="checkbox"/> 0 <input type="checkbox"/> 1 <input type="checkbox"/> 2 |
| Acute change or fluctuating course        | No = 0; Yes = 4                                                      | <input type="checkbox"/> 0 <input type="checkbox"/> 4                            |

**4AT Total Score:** \_\_\_\_\_ (0 = delirium unlikely; 1–3 = possible cognitive impairment; ≥4 = possible delirium — escalate per Section 4 below)

## 2.2 CAM (Preferred in Stroke — Best Validated Against DSM Criteria)

Positive CAM = Features 1 AND 2, plus either Feature 3 OR Feature 4:

|                          | Feature                              | Present? (interpret against reference exam)                 |
|--------------------------|--------------------------------------|-------------------------------------------------------------|
| <input type="checkbox"/> | 1. Acute onset or fluctuating course | <input type="checkbox"/> Yes<br><input type="checkbox"/> No |
| <input type="checkbox"/> | 2. Inattention                       | <input type="checkbox"/> Yes<br><input type="checkbox"/> No |
| <input type="checkbox"/> | 3. Disorganized thinking             | <input type="checkbox"/> Yes<br><input type="checkbox"/> No |
| <input type="checkbox"/> | 4. Altered level of consciousness    | <input type="checkbox"/> Yes<br><input type="checkbox"/> No |

CAM Result: ☐ Positive (1+2, and 3 or 4) ☐ Negative

## 3. Shift-by-Shift Screening Log

Complete at each required interval per protocol Section 5.3 (every 12 hours if high-risk / neuro-ICU / first 48h post-thrombectomy; every 24 hours if standard risk; and immediately with any acute change).

| Date / Time | Shift | Tool Used | RASS | Score | Positive? | Attributable to baseline deficit? (Y/N) | RN Initials | Provider Notified (time) / Action |
|-------------|-------|-----------|------|-------|-----------|-----------------------------------------|-------------|-----------------------------------|
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |
|             |       |           |      |       |           |                                         |             |                                   |

## 4. Positive Screen — Immediate Actions (Protocol Sections 6–7)

### Do Not Assume Delirium First

For any positive screen or acute change, notify the stroke team within the shift and complete the differential work-up before managing as delirium alone: STAT non-contrast head CT if new/worsening neuro deficit is suspected; consider STAT EEG if unexplained fluctuating unresponsiveness (rule out non-convulsive seizure); check for infection (aspiration pneumonia, UTI), metabolic derangement (glucose, sodium — especially with SAH), hypoxia, and review medications for deliriogenic or newly added agents.

| Action                                                                | Completed (initials / time) |
|-----------------------------------------------------------------------|-----------------------------|
| Notified bedside RN / charge nurse                                    |                             |
| Notified stroke team (resident / APP / attending)                     |                             |
| STAT head CT ordered if new/worsening deficit suspected               |                             |
| EEG considered if fluctuating unresponsiveness / seizure suspicion    |                             |
| Infection work-up initiated if indicated                              |                             |
| Metabolic panel / glucose / sodium checked                            |                             |
| Medication review completed (pharmacy notified if needed)             |                             |
| Non-pharmacologic prevention bundle initiated (Section 8 of protocol) |                             |
| Family notified / engaged at bedside                                  |                             |

## 5. Resolution

| <input type="checkbox"/> Two consecutive screens negative or stable relative to reference exam <input type="checkbox"/><br>date/time resolved: _____ |  |
|------------------------------------------------------------------------------------------------------------------------------------------------------|--|
| RN signature: _____<br>Provider co-signature (if applicable): _____                                                                                  |  |